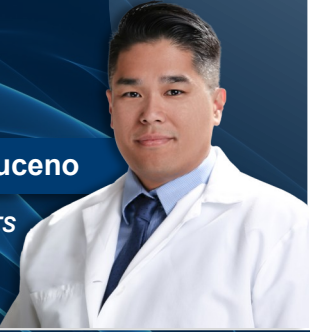




Shoulder Pain in Virtual Care

Clinical Case Quiz

A 47-year-old right-handed male, a recreational tennis player, presents via telehealth with 3 weeks of right lateral shoulder pain. No acute trauma. The pain worsens with overhead reaching and serving. No fevers or systemic symptoms.

Virtual exam:

- No swelling, erythema, or deformity
- Pain with abduction above 90°
- Mild limitation with external rotation
- Negative cross-arm test
- Can reach opposite shoulder

Which of the following is the most likely diagnosis?

- a. Acute anterior shoulder dislocation
- b. Rotator cuff tendinopathy/impingement
- c. Septic arthritis of the shoulder
- d. Proximal humerus fracture



The answer is b.

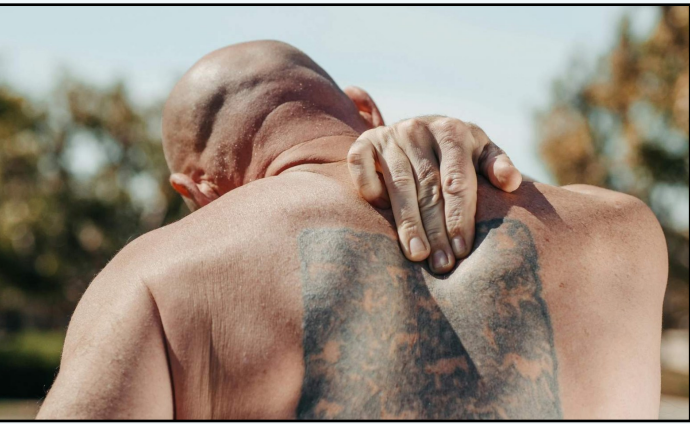
Virtual Care Focus: Evaluation of Shoulder Pain

Shoulder pain is a common complaint in primary care and urgent care, and virtual visits are increasingly the first point of contact. A structured approach can help clinicians differentiate urgent conditions, guide initial management, and determine when an in-person evaluation is needed.

1. Key History in Telehealth

Focus on onset, mechanism, and red flags:

- Mechanism of injury: acute trauma vs. gradual onset
- Pain location: lateral (rotator cuff, deltoid), anterior (biceps tendon, AC joint), posterior (scapular/referred pain from neck)
- Functional limitations: inability to lift arm, dress, reach overhead
- Systemic symptoms: fever, night sweats, weight loss (infection/malignancy red flags)
- Prior history: dislocations, prior surgery, repetitive occupational/sport activity



2. Virtual Exam Techniques

Patients can perform guided maneuvers on camera:

- Inspection: check for asymmetry, swelling, bruising, scars, erythema
- Active ROM: ask patient to flex, abduct, externally/internally rotate
- Pain-limited movement: can suggest rotator cuff pathology or impingement
- Cross-arm test: patient adducts across chest → pain = AC joint involvement
- Hand-behind-back reach: internal rotation assessment
- Functional test: “Can you reach to touch the opposite shoulder?”

Tip: Use the unaffected shoulder as a comparison.

3. Red Flags → Immediate In-Person Referral

- Acute trauma with suspected fracture or dislocation (visible deformity, inability to move arm)
- Neurovascular compromise (numbness, weakness, cool extremity)
- Signs of infection (fever, warmth, erythema, systemic illness)
- Inability to move arm after injury



4. Initial Virtual Management

- Analgesia: NSAIDs or acetaminophen as tolerated
- Activity modification: avoid aggravating movements, use of sling briefly if severe pain
- Home exercises: pendulum swings, gentle ROM if atraumatic and not red-flag. Can include handout with rehab exercises in patient instructions.
- Imaging referral: recommend in-person evaluation prior to imaging
- Follow-up: 1–2 weeks to reassess function and pain

5. Virtual Carer’s Role

- Efficient triage: quickly distinguishes minor strain vs. urgent conditions
- Reassurance: reduces unnecessary ER visits for atraumatic pain
- Continuity: enables early physical therapy/rehab exercises and coordinated care

Key Takeaways for Clinicians

- Comprehensive history-taking is the cornerstone in virtual visits
- Guided ROM and functional maneuvers are feasible over virtual encounters
- Always screen for trauma, infection, or neurologic compromise

References

American Academy of Orthopaedic Surgeons. (2025, August 28). AAOS updates clinical practice guideline for the management of rotator cuff injuries. American Academy of Orthopaedic Surgeons. <https://www.aaos.org/aaos-home/newsroom/press-releases/aaos-updates-clinical-practice-guideline-for-the-management-of-rotator-cuff-injuries/>

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